

NURSING: HOW WE THINK ABOUT VALUE

A transparent methodology for calculating return on investment

THE CONTEXT

Nursing is the hardest setting to model ROI--and the most important to get right. In outpatient medicine, a physician saves 4 minutes per visit, and you can trace a path to wRVU lift or capacity expansion. Nurses don't bill. They don't generate wRVUs. And yet nursing documentation burden is massive--25-35% of every shift spent on flowsheets, assessments, handoffs, and charting. So where does the value live?

TWO VALUE CATEGORIES

LABOR ECONOMICS

Overtime, retention, and agency spend. These are real dollars that show up in the budget. When nurses spend less time documenting, they finish shifts on time, burn out less, and the organization needs fewer expensive travel nurses.

Direct, measurable value

CARE QUALITY ENABLEMENT

Falls, pressure injuries, patient satisfaction. These outcomes are influenced by bedside time. More time caring, less time charting, better outcomes. But the causal chain is indirect--documentation supports care, it doesn't replace it.

Potential value (shown separately)

OUR APPROACH: We model both categories--but we're honest about what's directly measurable versus what we only enable. Labor economics value is defensible in CFO conversations. Care quality potential is real but requires clinical practice to realize.

THE HONEST LIMITS

[+] WHAT WE MEASURE

Direct attribution

- OT hours pre/post
- Doc time per shift
- Shift completion rates
- Chart completion timing

[~] WHAT WE INFLUENCE

Indirect attribution

- Turnover rates (one factor)
- Agency utilization
- Nurse satisfaction
- Burnout indicators

[] WHAT WE ENABLE

Supportive only

- Falls prevention
- HAPI prevention
- HCAHPS improvement
- Clinical outcomes

CONNECTED VALUE

When nurses document thoroughly and in real-time, it directly impacts inpatient revenue. Complete nursing documentation creates a clinical picture that supports accurate coding and stronger appeals. Nursing assessments capture clinical indicators (CC/MCC) that support accurate DRG assignment, and provide contemporaneous evidence for payer appeals. These benefits are quantified in the Inpatient Setting methodology.

VALUE MECHANISMS

OVERTIME REDUCTION

DIRECTLY MEASURABLE

MECHANISM

When nurses spend less time documenting, they complete shifts on time. Not all time saved becomes OT reduction--some goes to care--but a measurable portion does.

CALCULATION

Hours saved x Conversion rate (15-40%) x OT rate (1.5x base) = OT Savings

RETENTION SAVINGS

INDIRECTLY ATTRIBUTABLE

MECHANISM

Documentation burden is a top driver of nurse burnout. Burnout drives turnover. Reducing burden can help retain nurses who would otherwise leave.

CALCULATION

FTEs x Turnover x Burnout% x Abridge impact (10-25%) x Replacement cost

AGENCY LABOR REDUCTION

CONNECTED TO RETENTION

MECHANISM

When nurses leave, hospitals fill gaps with agency/travel nurses at 2-3x the cost. Improved retention directly reduces premium labor dependency.

CALCULATION

Nurses retained x Weeks of coverage (8-16) x Weekly premium (\$2-4K)

HAPI PREVENTION

POTENTIAL VALUE

MECHANISM

Real-time documentation ensures skin assessments, turning schedules, and risk factors are captured--enabling earlier intervention for pressure injuries.

CALCULATION

Current HAPIs x Doc-preventable rate (5%) x Cost per HAPI (\$10-50K)

FALLS PREVENTION

POTENTIAL VALUE

MECHANISM

Real-time documentation ensures fall risk scores, mobility status, and environmental factors are captured--enabling earlier preventive action.

CALCULATION

Current falls x Doc-preventable rate (5%) x Cost per fall (\$3-30K)

Note: CMS does NOT reimburse for hospital-acquired fall injuries.

PATIENT EXPERIENCE (HCAHPS)

QUALITATIVE

MECHANISM

When nurses spend less time documenting, they spend more time with patients. Research shows bedside time correlates with satisfaction scores.

CONSIDERATION

HCAHPS is influenced by dozens of factors. Track as directional indicator.

Top quartile HCAHPS = ~2% higher reimbursement via VBP.

WHY "POTENTIAL" VALUE: The link between documentation and care quality is indirect. We don't cause fewer falls--we enable the visibility that helps prevent them. Clinical practice matters more than documentation.

KEY ASSUMPTIONS

Every number has a source. We don't hide assumptions--we highlight them so you can adjust based on your reality.

Assumption	Range	Default	Source
Time saved per shift	15-30 min	20 min	Abridge customer data
OT conversion rate	15-40%	25%	Implementation studies
Nurse turnover rate	15-25%	18%	NSI Nursing Solutions 2024
Burnout-related turnover %	30-50%	40%	ANA research
Abridge retention impact	10-25%	15%	Conservative estimate
Nurse replacement cost	\$40-65K	\$52K	NSI 2024 Report
Agency coverage weeks	8-16 wks	12 wks	Industry average
Weekly agency premium	\$2-4K	\$2,500	Travel nurse rates
Doc-preventable HAC rate	3-8%	5%	Conservative estimate
Cost per HAPI	\$10-50K	\$26K	AHRQ data
Cost per fall with injury	\$3-30K	\$6.5K	CMS data

WHY WE DEFAULT CONSERVATIVE: *We'd rather show a smaller number you can defend than a larger number that falls apart under scrutiny.*

VALIDATION PATH

Our projections are starting points. The real answers come from your data.

TIME SAVINGS

Before:

- Survey nurses on doc time/shift
- Review EHR session data

After:

- Repeat measurements
- Compare with utilization data

Timeline: 2-4 weeks

OVERTIME REDUCTION

Before:

- Baseline OT hours per unit/month
- Note seasonal patterns

After:

- Track OT on Abridge vs. control
- Control for census/acuity changes

Timeline: 2-3 months

RETENTION

Before:

- Baseline turnover by unit
- Exit interview data on burnout

After:

- Track turnover Abridge vs control
- Survey on burnout/doc satisfaction

Timeline: 12-18 months

CARE QUALITY

Before:

- Baseline HAPI/falls rates by unit
- Baseline HCAHPS scores

After:

- Track rates Abridge vs control
- Be cautious about attribution

Treat quality as bonus, not promise